



PTH 6135: Course Title: Patient / Client Management III

Spring 2026

Monday and Wednesdays 8-1

3 different modules

Course Information:

Semester: Spring 2026

Credit Hours: 4

Weekly Clock Hours: 6

Lecture: 3

Lab: 3

Semester Clock Hours: 90

Lecture: 45

Lab: 45

Instructor:

Spine Module Instructor and Rounds leader

Janine DeBaets PT, DPT, OCS

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Office hours: Mondays 9-10 & Wednesday 1:30- 3:30 & by appointment

UE Module Instructor:

Office hours: By appointment only

Kaylee Pobocik PT, DPT

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LE Module Instructor

Office: SRH 201

Stephanie Sprout PT, DPT

Office hours: Monday and Wednesdays 12-1:30& by appointment

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Pre Requisites: Patient / Client Management II, Integrated Clinical Experience II

Co Requisites: Pathophysiology & Pharmacology I, Exercise Prescription & Nutrition, Movement Systems, Dynamic Systems II, Integrated Clinical III

Course Description Patient / Client Management III is a problem-based course with tutorial and laboratory sessions and continues to develop depth and problem solving. This course specifically adds evaluation and interventions for musculoskeletal specific pathologies, neuroscience to previously established A&P causal reasoning, and the hierarchy of adaptation to consideration of intervention effects.

Student Learning Outcomes

1. Implements appropriate teaching methods commensurate with the needs of the learner. (7D12)
2. Explain screening and the genetic (through history), physiological, anatomical and musculoskeletal factors that contribute to determining whether patients/clients need further examination or consultation by a physical therapist or referral to another health care professional (7D16)
 - a. Identify and refer clients who might benefit from services or from consultation with other professionals for management of issues related to a diagnosis and be able to effectively explain reasons for and benefits of services (including genetic services)
3. Obtain a history and relevant information from the patient/client and from other sources as needed including a genetic family history with a minimum three generation history for patients with musculoskeletal conditions. (7D17)
4. Select and perform physical therapy tests and measures and identify factors that influence their use based on patients age; diagnosis and health status to assess patients / clients with primary musculoskeletal conditions, including Anthropometric Characteristics, Assistive Technology, Balance, Circulation (deep vein thrombosis, pulse, venous stasis), Environmental Factors, Gait, Joint Integrity and Mobility, Motor Function, Muscle Performance, Pain, Posture, Range of Motion, Reflex Integrity, Sensory Integrity, and

Skeletal Integrity with complex musculoskeletal cases in a structured variable clinical practice condition. (7D19)

5. Evaluate data from the examination (history; health record; systems review; and tests and measures) to make clinical judgments for patients with musculoskeletal conditions. (7D20)
6. Identify components and their interactions of the International Classification of Function (ICF) to describe a patients/clients impairments; activity and participation limitations for patients with musculoskeletal conditions. (7B, 7D21)
7. Determine a diagnosis in patients with musculoskeletal conditions that guides future patient/client management. (7D22)
8. Determine goals and expected outcomes for patients with musculoskeletal conditions within available resources (including applicable payment sources) and specify expected length of time to achieve the goals and outcomes. (7D23)
9. Develop a safe and effective plan of care for patients with musculoskeletal conditions that is developed in collaboration with appropriate stakeholders; including patients/clients; family members; payors; other professionals and other appropriate individuals. (7D24)
10. Determine the components of the plan of care for patients with musculoskeletal conditions that may; or may not; be directed to the physical therapist assistant (PTA) based on (a) the needs of the patient/client; (b) the role; education; and training of the PTA; (c) competence of the individual PTA; (d) jurisdictional law; (e) practice guidelines policies; and (f) facility policies. (7D25)
11. Create a discontinuation of episode of care plan (D/C NOTE) for patients with musculoskeletal conditions that optimizes success for the patient in moving along the continuum of care. (7D26)
12. Select and perform physical therapy interventions to achieve patient/client goals and outcomes in patients with primary musculoskeletal conditions. Interventions may include: Assistive Technology: Prescription; Application; and; as appropriate; Fabrication or Modification, Biophysical Agents, Functional Training in Self-Care and in Domestic; Education; Work; Community; Social; and Civic Life, Integumentary Repair and Protection Manual Therapy Techniques (including mobilization/manipulation and non-thrust techniques), Motor Function Training (balance; gait; etc.), Patient/Client education, Therapeutic Exercise in complex cases and structured variable clinical practice environments. (7D27)
13. Manage the delivery of the plan of care that is consistent with professional obligations; inter professional collaborations; and administrative policies and procedures of the practice environment for patients with musculoskeletal conditions. (7D28)
14. . Delineate, communicate and supervise those areas of the plan of care that will be directed to the PTA (7D29)

15. Monitor and adjust the plan of care in response to patient/client status for patients with musculoskeletal conditions. (7D30)
16. Assess patient outcomes; including the use of appropriate standardized tests and measures that address impairments; functional status and participation for patients with musculoskeletal conditions. (7D31)
17. Complete accurate documentation that follows guidelines and specific documentation formats required by state practice acts; the practice setting; and other regulatory agencies for patients with musculoskeletal conditions. (7D32)
18. Describe the provision of care through direct access for patients with musculoskeletal conditions. (7D35)
19. Describe the case management process for patients with musculoskeletal conditions. (7D36)
20. Assess and document safety risks of patients and the healthcare provider and design and implement strategies to improve safety in the healthcare setting as an individual and as a member of the interprofessional healthcare team. (7D37)

Required Materials:

- Access Physiotherapy including the FA Davis Collection <https://fadavispt.mhmedical.com/>
- Access Physiotherapy <https://accessphysiotherapy.mhmedical.com/>
- Subscription to Physio U required for this course <https://app.clinicalpattern.com/signin/?redirectTo=/>

- APTA Clinical Practice Guidelines <https://www.apta.org/patient-care/evidence-based-practice-resources/cpgs>

- APTA PT NOW <https://www.apta.org/patient-care>

- Kaltenborn, F. M. (1989). Manual Mobilization of the Joints: The Extremities (Vol. 1 & 2). Oslo, Norway: Olaf Norlis Bokhandel. Most updated edition

- Wise CH. *Orthopaedic Manual Physical Therapy: From Art to Evidence*. Burlington, MA: Jones & Bartlett Learning; Most updated edition
<https://fadavispt.mhmedical.com/book.aspx?bookID=2154#164750737>

Description of teaching methods and learning experiences

Tutorial sessions, videos and guided learning activities, laboratory, group discussions, case presentations

Assignments and Activities

Post Modules Exams (3) 45 %

Final cumulative exam 25%

Cases(SCRIPTs prep) Professional behavior and expectation.

Rounds 10% (25% attendance, 75% case presentation)

OSCE 20%

Exams

Case based exam at the end of each Module for a total of 3 – worth 15% each

Final Cumulative exam at the end of the semester on all Modules- Worth 25%

Cases

Throughout each module students will be presented with several cases (macro and micro case problem sets) and will submit responses to specific questions and problems prior to participating in class discussion. The case problem sets in PCM III specifically focus on evaluation and interventions for musculoskeletal specific pathologies, neuroscience, anatomy and physiological causal reasoning, and the hierarchy of adaptation to consideration of intervention effects.

Rounds

A weekly discussion, augmenting case content. See additional information on Canvas site.

Objective Structured Clinical Examination (OSCE)

For general background see the DPT Student Handbook; for specific content for PCM III OSCE see the OSCE III Content as part of this syllabus. Note, content for the OSCE is an integration of material from all courses in the semester with PCM, along with a cumulative expectation from all prior courses.

OSCE Failure

OSCE grades are incorporated into the PCM course grades. Successfully completing each OSCE requires a grade of at least a 73%. An OSCE grade of less than 73% results in a PCM course grade of an F. There are elements to an OSCE grade that are considered safety concerns and failure in an element related to a safety concern can result in immediate failure of the OSCE. If a student fails an OSCE, he/she can repeat one time and achieve a maximal grade of a 73%. Prior to the repeat OSCE the student must meet with the PCM course coordinate to review the OSCE results, develop a remediation plan and make plans for the repeat OSCE. The repeat OSCE will be a subset of the full OSCE, including only the elements that clearly led to failure.

Student Evaluation/Grading

A	94.0 – 100
A-	90.0 – 93.9
B+	87.0 – 89.9
B	83.0 – 86.9
B-	80.0 – 82.9
C+	77.0 – 79.9
C	73.0 – 76.9
F	below 73.0

Students are reminded that continuation in the program necessitates achieving an overall grade point average of 3.0 (B) or better. Grades below 2.0 (C) are not acceptable or counted toward the DPT degree. No more than 9 credits below 3.0 may be counted toward the DPT degree (grades of B-, C+, or C). Make-up examinations will be given only in specific cases of absence (documented absences). Circumstances that may warrant an absence should be discussed with the instructor before the scheduled time of the exam. Failure to notify the instructor as stated may result in a zero for the exam.

Professional Conduct: Students are reminded of the program expectation that they follow Professional Conduct, see the Student Handbook, Professional Conduct and Reasonable Professional Expectations for the process of handling violations, remediation, and consequences. Violations of Professional Conduct (such as unexcused absences or failure to complete assignments or prepare for class discussions) will result in a verbal warning. Continued violation will result in a written notification of a grade reduction of up to 10%. Violations of Professional Conduct that reach an egregious level, such as cheating on a quiz or exam, will result in immediate failure of that quiz or exam.

Cell Phones: Cell phone use (including texting) is prohibited in class. Also, please respect your classmates by turning your phone to vibrate prior to class start time. Do not turn phones off as the University uses texts to relate emergency information.

Graduate Academic Policies: The DPT Program is a selective admissions cohort-structured program. Some Graduate Academic Policies may not be relevant to the program. Students should refer to the Doctor of Physical Therapy Program Handbook for specific information about Scheduling, Capstones, and Internships. The sections on Grading System and Academic Standing Policy do apply to DPT students: <https://coursecatalog.plymouth.edu/university-policies-procedures/graduate-academic-policies/>

Fair Grading Policy: Fair and equitable grading reflects values to which all members of the Plymouth State community commit themselves. Grades are used to assess the relative extent to which students achieve course objectives in all for-credit courses at PSU. Academic freedom allows instructors to determine course objectives within the bounds of established curricula, and the means by which a student's mastery of those objectives will be evaluated; and evaluate the quality of work on individual exams or assignments. Information can be found at <https://coursecatalog.plymouth.edu/university-policies-procedures/> under Fair Grading.

Student Support: Student Support Foundation (SSF) provides short-term emergency financial assistance and long-term student support. For more information, see: <https://campus.plymouth.edu/student-support-foundation/>

Student Support Foundation also runs a food pantry, located in Belknap Hall. To learn more about SSF or access the food pantry, either via open hours or a private appointment, contact the SSF advisor, at psu-ssf@plymouth.edu.

Academic Dishonesty: Cheating and plagiarism are very serious forms of academic dishonesty. Any use of unauthorized assistance on exams, papers, homework assignments, or other course work constitutes cheating. Knowingly providing assistance during an exam or allowing other students to copy one's work is also a serious form of academic dishonesty. Plagiarism consists of submitting written work that has been developed wholly or partially by someone else. Submitting written work in which the ideas of others have been duplicated or even paraphrased without proper reference to the author is also a form of plagiarism. Also considered plagiarism is the acquisition of term papers or other assignments from another source and subsequent presentation of these materials as the student's own work. In addition, students may not use papers in more than one course without the permission of both instructors.

Any student suspected of academic dishonesty will be reported to the Program Director immediately. A full description of academic integrity, violations and procedures when a violation to academic integrity in a course is suspected can be found at:

<https://coursecatalog.plymouth.edu/university-policies-procedures/> under Academic Integrity.

Attendance Policy: Attendance is expected to all class sessions. Students are ultimately responsible for their attendance, however faculty will assess students' Professional Expectations based, in part, on their attendance (See Student Evaluation/Grading). Students who miss class for any reason are responsible for the information and work missed. Students should speak with the professor before missing the scheduled class when able.

Campus Accessibility Policy: Plymouth State University is committed to providing students with documented disabilities equal access to all university programs and facilities. If you think you have a disability requiring accommodations, you should immediately contact Campus Accessibility Services (CAS) in Speare 210 (535-3300) to determine whether you are eligible for such accommodations. Academic accommodations will only be considered for students who have registered with CAS. If you have a Letter of Accommodation for this course from CAS, please provide the instructor with that information privately so that you and the instructor can review those accommodations: <https://campus.plymouth.edu/accessibility-services/>

LibGuide for Physical Therapy:

The Libguide for Physical Therapy is an information sharing platform created through a collaboration between the library and the DPT faculty that presents relevant resources to support students in the Physical Therapy program. Examples of information you will find include tutorials to better understand citing sources in APA and AMA style, resources on how to avoid plagiarism, quick access to relevant databases and interlibrary loan as well as a contact link to the programs' assigned librarian. To view the DPT Program Libguide go to <https://library.plymouth.edu/physical-therapy>.

Diversity and Inclusivity Statement:

We consider this classroom to be a place where you will be treated with respect, and we welcome individuals of all ages, backgrounds, beliefs, ethnicities, genders, gender identities, gender expressions, national origins, religious affiliations, sexual orientations, ability - and other visible and non-visible differences. All members of this class are expected to contribute to a respectful, welcoming and inclusive environment for every other member of the class. See this web page for the Plymouth State University IDEA Center Inclusion, Diversity, Equity, and Access.

<https://www.plymouth.edu/idea-center>

Specific outline of content

Each module is a 4-week session (25 hours of direct contact) including 2 hours per week of tutorial and 4 hours per week of laboratory and a one-hour "Rounds" tutorial. Students are encouraged to use labs in their non class time as frequently as possible to practice their skills.

Students are broken into 3 groups and rotate through the modules - so the below order is not the same for all students. Rounds offer a common time shared between the three groups of students for case presentations. Students in each module are encouraged to work with one another throughout the term.

Each module has up to 10 students and three scheduled sessions per week. Two sessions are 3 hours, and one session is common to all modules for 1 hour (Rounds). Each scheduled session has access to both the classroom and labs so that time between the two can vary week to week based on topics being covered.

There are 4-6 cases per week for each module and students will submit weekly responses to case problem sets in addition to discussions with faculty and peers during the tutorials and labs. Of the cases per week at least 2 will be unfolding over 2 weeks (macro cases) and others will be new each session (micro cases).

Tutorial sessions emphasize reasoning through the PCM process and helping students with problem solving and acquiring necessary resources.

Lab sessions emphasize patient care / psychomotor skills for examination and intervention techniques.

Lower Extremity Module

Given primarily lower extremity based case problems students will identify, perform and evaluate physical therapy patient / client management including but not limited to:

Environmental Factors (stairs, etc)

Mobility (including Locomotion)

Muscle Performance (Including Lab sessions)

Assistive Technology: Prescription; Application; and; as appropriate Fabrication or Modification (Splinting) (Including Lab sessions)

Functional Training (Including Lab sessions)

Manual Therapy Techniques (Including Lab sessions)

Motor Function Training (Including Lab sessions)

Patient/Client education (Including Lab sessions)

Therapeutic Exercise (Including Lab sessions)

Each lab session includes:

- Instruction and supervised practice of manual techniques
- Instruction and supervised practice of special tests/examination techniques
- Supervised practice of combination interventions (assistive technology, biophysical agents, functional training, integumentary repair and protection, manual therapy (MT), motor function training, patient/client education, therapeutic exercise

Tentative Lab Emphasis:

Sessions 1 - 2: Hip interventions, MT & special tests

Sessions 3 - 5: Knee interventions, MT & special tests

Sessions 6 - 8: Foot/Ankle interventions, MT & special tests

Upper Extremity Module

Given primarily upper extremity based case problems students will identify, perform and evaluate physical therapy patient / client management including but not limited to:

Environmental Factors

Mobility (including Locomotion)

Muscle Performance (Including Lab sessions)

Assistive Technology: Prescription; Application; and; as appropriate Fabrication or Modification (Splinting) (Including Lab sessions)

Functional Training (Including Lab sessions)

Manual Therapy Techniques (Including Lab sessions)

Motor Function Training (Including Lab sessions)

Patient/Client education (Including Lab sessions)

Therapeutic Exercise (Including Lab sessions)

Each lab session includes:

- Instruction and supervised practice of manual techniques
- Instruction and supervised practice of special tests / examination techniques
- Supervised practice of combination interventions (assistive technology, biophysical agents, functional training, integumentary repair and protection, manual therapy (MT), motor function training, patient/client education, therapeutic exercise

Tentative Lab Emphasis:

Sessions 1 - 4: Shoulder interventions, MT & special tests, Peripheral nerve entrapments

Sessions 5: Elbow interventions, MT & special tests, Peripheral nerve entrapments

Sessions 6 - 8: Wrist / hand interventions, MT & special tests, Peripheral nerve entrapments

Spine & Thorax Module

Given primarily spine based case problems students will identify, perform and evaluate physical therapy patient / client management including but not limited to:

Environmental Factors

Mobility (including Locomotion)

Muscle Performance (Including Lab sessions)

Functional Training (Including Lab sessions)

Manual Therapy Techniques (Including Lab sessions)

Motor Function Training (Including Lab sessions)

Patient/Client education (Including Lab sessions)

Therapeutic Exercise (Including Lab sessions)

Each lab session includes:

- Instruction and supervised practice of manual techniques
- Instruction and supervised practice of special tests / examination techniques
- Supervised practice of combination interventions (assistive technology, biophysical agents, functional training, integumentary repair and protection, manual therapy (MT), motor function training, patient/client education, therapeutic exercise

Sessions Lab Emphasis:

Sessions 1 - 4: Lumbosacral interventions, MT & special tests

Sessions 5: Thoracic interventions, MT & special tests

Sessions 6 - 8: Cervical interventions, MT & special tests

Tentative Class Schedule:

In summary – Section 01 rotates through UE, LE, Spine (in that order)

Section 02 rotates through LE, Spine, UE (in that order)

Section 03 rotates through Spine, UE, LE (in that order)

ski day Feb 11-no class

Spring break March 16-20

Finals week May 4-8

Module	Section 01	Section 02	Section 03
Upper Extremity (8:00-11:00) Kaylee Pobocik	1/20 – 2/18	3/30- 4/22	2/23 – 3/25
Lower Extremity (9:00-12:00) Stephanie Sprout	2/23 – 3/25	1/20 – 2/18	3/30- 4/22
Spine (10:00-1:00) Janine DeBaets	3/30- 4/22	2/23 – 3/25	1/20 – 2/18

Post Module Examinations:

- Post Module Exam 1: Friday, Feb 20 8 AM – 9:30 AM
- Post Module Exam 2: Friday, March 27 8 AM – 9:30 AM
- Post Module Exam 3: Friday, April 24 8 AM- 9:30 AM

Final cumulative Exam 4: May 4 at 9 AM-11 AM

- OSCE during final exam week: May 6^h at 8 AM

General Case Organization within a Module

Week	Monday	Wednesday	Friday (Rounds): 9-10 am
1	Macro Case A1 Micro Case 1	Macro Case A2 Micro Case 2	Case Presentations
2	Macro Case A3 Macro Case B1	Macro Case A4 Macro Case B2	3 of these Fridays will include

	Micro Case 3	Micro Case 4	Post Module Exams (see above)
3	Macro Case B3 Macro Case C1 Micro Case 5	Macro Case B4 Macro Case C2 Micro Case 6	
4	Macro Case C3 Micro Case 7	Macro Case C4 Micro Case 8	

Macro cases

3 in total per module, A, B & C

These cases are 2 weeks in length (4 sessions); their primary purpose is to get you thinking about the time dynamics of patient care. Just because they are included in the course for 2 weeks does not mean they are limited to “real time” two weeks. In other words, the two weeks of the term could be any time frame in real time depending on how the case is organized.

Micro cases

At least 8 per module, 1 – 8 (could be up to 12, with additional Micro cases in weeks 1 and 4)

These cases are for a single session but can be at any point in the continuum of care. In other words, they can be for teaching about examination (what to pick, how to do it, etc); or evaluation – where all the examination data is presented, and they need to summarize and evaluate including intervention; or mixed for intervention where a mix of data is provided with students identifying what they need to know to move forward and how the information once obtained would be useful in moving forward.

